Getting Your Wife's Care Requirements on Paper

A step-by-step plan before you compare settings, with a line for who does each step and by when.

The goal: before anyone compares settings, you can hand her care team one page that says what help your wife gets, what has been confirmed, what is essential, and which questions are still open.

Share the list. Put a name and a day next to each step, and tick it when it's done.

ACTION	WHO	BY WHEN	DONE
1. Read through what you already have: your earlier list of her needs and gaps, or your own notes on a day with her.			
2. List the tasks she gets help with, one per line, in plain words.			
3. Beside each task, write when the help happens, only where you've seen it or a professional has documented it. Everywhere else, write “not confirmed.”			
4. Write down how she gets around and how she communicates, and what helps with each.			
5. Write your questions about supervision as questions. Leave the answers to the people who assess her.			
6. Ask her care team which clinical capabilities they say are required. Copy their words, with the name and the date.			
7. Ask each person in the family what they can realistically keep doing, and write down what they say.			
8. Sort your list into essentials and preferences. Put anything medical, about her medications or about supervision under “not sure.”			
9. Next to each open question, write who it goes to: her doctor, her nurse, her therapist, the social worker or your care consultant.			

You're done when

The help she gets is written as plain tasks. What her care team has confirmed is marked as confirmed, with a name and a date. Your essentials and preferences are sorted, and every open question has a name beside it. What comes after that, the review of those questions, any word on the kind of care she needs and the comparing of settings, isn't on this list, because it isn't the family's to do alone.

If the days get away from you

- Don't fill a gap by guessing. “Not confirmed” is a finished line.

- If the social worker asks for your thoughts before the list is done, bring what you have. A half-finished page, with its open questions marked, is still worth reading from.
- If things feel hurried, the Family Caregiver Alliance says it is reasonable to ask the discharge planning team to slow things down so nothing is overlooked.

What this plan doesn't do

It organizes your part. It doesn't certify that any arrangement is safe for your wife, decide what care she needs, or stand in for the professional review it prepares you for.

Ready for the next step?

You've put your part in order, one step at a time. The care interview goes through your wife's daily life area by area with you, so the parts that don't show up at home are covered too, and turns your answers into a Family Care Profile you can bring to any care conversation.

Create Your Free Family Care Profile ›

WHERE THIS COMES FROM

The note on asking the discharge planning team to slow things down follows the Family Caregiver Alliance's guide. The nine actions come from the Family Care Roadmap's own working list for this step.

1. Family Caregiver Alliance, Hospital Discharge Planning: A Guide for Families and Caregivers

From the Family Care Roadmap · Richard Ladson, Certified Medicaid Planner. Draft for review, generated 2026-10-04.